

ICD-0-3

PATIENT HISTORY:

The patient is a year old man.
PRE-OP DIAGNOSIS: Bladder carcinoma.
POST-OP DIAGNOSIS: Same.
PROCEDURE: Radical cystoprostatectomy.

carcinoma urothelial 8120/3
Site: bladder, lateral wall c67.2

for 4/7/11

FINAL DIAGNOSIS:

PART 1:

LEFT DISTAL URETER, BIOPSY -

- A. SEGMENT OF URETER SHOWING BENIGN UROTHELIAL MUCOSA.
- B. NO EVIDENCE OF NEOPLASIA IS PRESENT.

PART 2:

RIGHT DISTAL URETER, BIOPSY -

- A. SEGMENT OF URETER SHOWING BENIGN UROTHELIAL MUCOSA.
- B. NO EVIDENCE OF NEOPLASIA IS PRESENT.

PART 3:

RIGHT PELVIC LYMPH NODES, EXCISION -

NO EVIDENCE OF NEOPLASIA IN 4 (FOUR) LYMPH NODES EXAMINED.

PART 4:

LEFT PELVIC LYMPH NODES, EXCISION -

NO EVIDENCE OF NEOPLASIA IN 9 (NINE) LYMPH NODES EXAMINED.

PART 5:

URINARY BLADDER AND PROSTATE, RADICAL CYSTOPROSTATECTOMY.

URINARY BLADDER:

- A. INVASIVE UROTHELIAL CARCINOMA, HIGH-GRADE (WHO/SUP), GRADE 3 OF 4 (ASH) WITH FOCI OF SQUAMOUS DIFFERENTIATION (see comment).
- B. THE CARCINOMA IS 5.5 CM IN GREATEST DIMENSION.
- C. THE UROTHELIAL CARCINOMA INVADES THROUGH THE MUSCULARIS PROPRIA INTO THE PERIVESICAL ADIPOSE TISSUE.
- D. NO EVIDENCE OF ANGIOLYMPHATIC OR PERINEURAL INVASION IS PRESENT.
- E. UROTHELIAL CARCINOMA IN SITU IS PRESENT.
- F. NON-NEOPLASTIC URINARY BLADDER MUCOSA SHOWS CYSTITIS CYSTICA.
- G. ALL SURGICAL MARGINS ARE FREE OF INVASIVE CARCINOMA. THE URETHRAL MARGIN SHOWS PROSTATIC TISSUE WITH HIGH-GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN), BUT WITHOUT EVIDENCE OF INVASIVE CARCINOMA.
- H. PATHOLOGIC TNM STAGING: pT3a NO Mx.
- I.

PROSTATE:

- J. THE PROSTATE SHOWS FEW FOCI OF HIGH-GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA AND FOCAL STROMAL AND GLANDULAR HYPERPLASIA, WITH NO EVIDENCE OF INVASIVE CARCINOMA.

COMMENT:

Part 3: Foci of low-grade and foci of high-grade urothelial carcinoma are seen. The foci of high-grade urothelial carcinoma show a high nuclear/cytoplasmic ratio and prominent nucleoli and in some areas the cells are fusiform, giving an overall sarcomatoid appearance. However, the ASH grading assigned to this lesion is 3 of 4 because the tumor lacks the enlarged bizarre nuclei that are the hallmark of a grade 4 lesion.

An interesting finding is the infiltration of the tumor nests by large number of neutrophils; the biological significance of such a phenomenon is not known.

UUID: B2E8ABBC-33FF-4889-AE29-34097296C839
TCGA-BT-A200-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
NIHAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):		
Reviewer Initials		
Date Reviewed		

QUALIFIED / DISQUALIFIED
for 4/7/11

SYNOPTIC - PRIMARY URINARY BLADDER/URETHRA TUMORS

- A. Location of primary tumor: 5
- | | | | |
|-------------------|---------------|-----------------|-----------------------|
| 1. Dome | 4. Right wall | 7. Ureter | 10. No residual tumor |
| 2. Anterior wall | 5. Left wall | 8. Urethra | |
| 3. Posterior wall | 6. Trigone | 9. Multicentric | |
- B. Procedure: 3
- | | |
|-----------------------|-----------------------|
| 1. Partial cystectomy | 3. Cystoprostatectomy |
| 2. Total cystectomy | 4. Other |
- C. Size (if multicentric, maximum dimension of the most deeply invasive or largest tumor): 5.5 cm
- D. Non-invasive neoplastic lesions: 4
- | | |
|-----------------------|-------------|
| 1. Papilloma | 4. Flat CIS |
| 2. Inverted papilloma | 5. Other |
| 3. Papillary TCCa | |
- E. Invasive neoplasia: 1
- | | |
|---|--|
| 1. Urothelial (Transitional cell) carcinoma | 6. Mixed urothelial carcinoma |
| 2. Squamous carcinoma | 7. Lymphoma/leukemia |
| 3. Adenocarcinoma | 8. Sarcoma |
| 4. Sarcomatoid carcinoma | 9. Metastasis/direct extension from a contiguous primary |
| 5. Small cell/neuroendocrine carcinoma | 10. Other |
- F. Grade: 3 (Ash for TCCa grades 1-4) / 3 (WHO for TCCa grades 1-3)/HG (WHO / ISUP (LMP, LG, HG))
- G. Non-neoplastic and other conditions: 1
- | |
|---|
| 1. Von Brunn's nests, cystitis cystica/glandularis, intestinal metaplasia |
| 2. Nephrogenic metaplasia/ adenoma |
| 3. Granulomatous cystitis |
| 4. Interstitial cystitis |
| 5. Inflammatory pseudotumors |
| 6. Carbuncle |
| 7. Malakoplakia |
| 8. Other |
- H. Margins of resection: 2
- | | |
|-------------|-------------|
| 1. Positive | 2. Negative |
|-------------|-------------|
- I. Angiolymphatic invasion: 2
- | | |
|--------|-------|
| 1. Yes | 2. No |
|--------|-------|
- J. Perineural invasion: 2
- | | |
|--------|-------|
| 1. Yes | 2. No |
|--------|-------|
- K. Mitotic activity: 1 per 10 high power fields
- L. Flat dysplasia in background: 1
- | | |
|--------|-------|
| 1. Yes | 2. No |
|--------|-------|
- M. Number of positive lymph nodes: 0
- N. Total number of lymph nodes examined: 13
- O. Extracapsular spread of lymph node metastases: N/A
- | | |
|--------|-------|
| 1. Yes | 2. No |
|--------|-------|
- P. Prostate/seminal vesicles (If cystoprostatectomy)
- | | | |
|--|--------|-------|
| P1. Involved by bladder neoplasm: <u>2</u> | 1. Yes | 2. No |
| P2. Concurrent primary invasive prostatic neoplasm: <u>2</u> | 1. Yes | 2. No |
| P3. High grade PIN: <u>1</u> | 1. Yes | 2. No |
- Q. TNM stage: T 3a N 0 M x

